YG Critical Care Unit

Invasive Procedure: Lumbar Puncture

Procedure date/time:	
Operator:	
Bedside assistant:	
Supervised? (Name/grade):	

Addressograph

Name

Hospital Number

Date of Birth

Completion of this form is a joint responsibility between operator and bedside nurse/assistant

BEFORE THE PROCEDURE	
Patient identity checked	☐
Appropriate consent (eg verbal/form 4)	☐
25G Pencilpoint needle; sample bottles; manometer	☐
No contraindications (see overleaf);	☐
Any brain imaging reviewed	☐
Medicines and coagulation checked (see overleaf)	☐
Allergies checked	☐

TIME OUT Verbal confirmation with team before start of procedure	
Team members introduced, roles assigned	☐
Position optimal	☐
Happy with what samples required (see overleaf)	☐
Confirm that operator will seek help if no success after 2 attempts	☐
Any concerns?	**STOP: GET HELP**

During the Procedure		
Sterile gloves/gown/hat/mask worn by operator	☐	
Chloraprep 0.5% applied and dried	☐	
Sterile field maintained with fenestrated drape	☐	
Local anaesthetic used (if appropriate)	☐	
Samples taken (see overleaf for guide)	☐	
Dressing applied	☐	
Patient position:		Location: (eg L3/4)
No of attempts:		Opening Pressure:
Appearance/comments:		

SIGN OUT	
Sharps disposed of	☐
Drugs (eg Local Anaesthetic) documented in medication chart	☐
Samples sent; pink micro sheet completed	☐
Adverse events documented in notes and Datix submitted (or no adverse event)	☐
Post-procedure handover to nursing staff	☐
Kit and trolley cleaned, re-kitted and stored: low stock items re-ordered	☐
Signed	
Print GMC/NMC	

GIG CYMRU NHS WALES

Bwrdd Iechyd Prifysgol Betsi Cadwaladr University Health Board

Ysbyty Gwynedd Anaesthetics Department

Adran Anaestheteg

Contraindications to Lumbar Puncture

1. Patient refusal
2. Local pressure sore or infection over lumbar area
3. Coagulopathy
   - Platelets below 40 or INR above 1.4
   - Aspirin is NOT a contraindication
   - Other antiplatelets/antithrombotics/coag abnormality:
     ⇒ Follow appropriate guidance from Association of British Neurologists [scan QR code, right]
     ⇒ Or speak to haematology
4. Evidence of raised ICP:
   - Abnormal GCS, seizures
   - Focal neurological deficit
   - Bradycardia with elevated BP
   - Abnormal respiratory pattern

If any suspicion of raised ICP, order a CT brain.

ABN Guideline Here

Sample Guidance

4 universal sample containers. Label 1-4
15-20 drops per bottle
Bottle 1: Micro for MC&S.
Bottle 2: Biochem for glucose, protein, oligoclonal bands.
Bottle 3: Micro for: Viral PCR (HSV, VZV etc); Toxoplasma; Cryptococcus.
Bottle 4: Biochemistry for xanthochromia (protect from light)
[In some cases a 5th bottle is needed for cytology: this will be in liaison with Haematology]

Also send blood samples for:

- Blood cultures if infection suspected
- Protein and glucose (should be taken at same time as LP)

Feed back on this form!
We want to know what you think
Scan the QR code below

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